

Diabetic Emergency

History – Known Diabetic

- Medications (Insulin, Metformin, Orinase, Micronase, etc.). When last taken? Usual dose? Missed dose?
- Last meal eaten?
- Illicit drug use or toxic ingestion?
- History of trauma?
- Onset and Duration?
- Seizure activity or fever?

Signs and Symptoms – Hypoglycemia

- Sudden decrease in mental status
- Bizarre or intoxicated behavior / appearance
- Cool, diaphoretic skin
- Hunger
- Anxiety or Combativeness
- Headache
- Tremors
- Faintness
- Seizures or coma

Signs and Symptoms – Hyperglycemia

- Blood glucose > 250 mg/dL
- Thirst
- Dyspnea
- Frequent Urination

Consider DKA

- Abdominal pain
- Altered Level of consciousness
- Deep rapid (Kussmaul's) respirations
- Nausea and vomiting

Differential Diagnosis:

- Head trauma, CNS (stroke, seizure, tumor, infection), thyroid problem, toxicologic, environmental exposure, hypoxia, hypotension, cardiac, psychiatric disorder

EMT/AEMT

- Follow General Medical Care Guideline
- Follow Adult Resuscitation Guideline, as indicated

Blood Glucose < 60 and Alert

- **Oral glucose** (if able to protect airway)

EMT/AEMT - Expanded Scope

- **D10W** 25g IO if patient is unable to tolerate oral glucose

Paramedic

Blood Glucose < 60 and Altered Mental Status OR NOT Alert

- **D10W** 25 g IV/IO (25g/250mL)

Repeat Blood Glucose < 60 after 5 minutes

- Repeat **D10W**

Blood Glucose >250 and Suspected DKA

- **NS/LR** 500 mL to 2000 mL IV/IO

If patient is taking oral diabetic medications, took too much insulin, or is not tolerating oral intake, the patient should be evacuated for observation and treatment in the hospital

- Monitor blood glucose every 30 minutes during evacuation if evacuating by foot
- Monitor blood glucose every 1 hour if evacuation by carryout